

العمر(تاريخ الميلاد) : 15-12-1970 المهنة : ربه منزل

التوقيع: (-----)

التعليق على التشخيص : -----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 نفقات :
 ملابحات :
 جهة العلاج :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

--

Name: حنان شعبان حسن عثمان National ID: 27012152602162	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Tamoxifen therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠١٩/٠٤

٢٧.١٢١٥٢١.٢١٦٢

رؤية متوزن

متوزجة

مسلمة

عشرى المراهق حسن

البلقة سارية حق ٢٠٢١/٠٣/٢٧



HY8598805

مملكة فلسطين الشعبية

حلق

شعبان حسن عطين

التحرير

مركز بئر - البيرة

٢٧٠١٢١٥٢١٠٢١٦٢

MDT FORM

31 / 12 / 2025 G

Name	حنا حبان	MRN	3027 / 2021
		ID	CV.191057.917
Age	55	Phone	01126777135
Menopausal status	Hot	Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	AI Shift
CO morbidity			

Clinical Data

BX:	IDC			R	L
ER:	PR:	HER 2:	Ki67:		
7/8	+	+	3		

Sonomammography:

2022: Rt breast + Ca

Mets Workup:

ER 7/8, Her2 + 3

NACT TCHP → Rt BCS + PCR

adjuvant Hercept 2022

Recommendations:

& Na on hormonal th
Tamoxifen Since then "2022

11 / 2025 → FSH 23, E2 14 "PostMen
for Shift to AI for 35y

MDT OUTCOME & PLAN

f. fenza "complete Zylall"
3-4y + Tamoxifen

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: حسان شعبان حسن		
MR. : 5082		
Detailed Medical Report		
		السيد
تحيه طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient has a history of right breast cancer treated with neoadjuvant chemotherapy (NACT) followed by right breast conservation surgery (Rt CBS) with a ypT0N0 pathological result in June 2022. Post-operative management included radiation therapy (PORT) and adjuvant endocrine therapy with tamoxifen. The most recent breast-specific monitoring was a breast-specific mammogram (BSM) performed in September 2025, which was free of disease, and a bone scan conducted in July 2025, also showing no metastatic activity.		
تحريرا في ,		
	التاريخ :	20-09-2026
	الوقت :	
	ندى حسن صلاح كامل	الطبيب المعالج :
يعتمد ,		

Patient Name : الأستاذ / حنان شعبان حسن عثمان
Patient No. : 27012152602162
Path No. : SR-392
Sex : Female
Age : 50 Y
Referring Doc :

Request No. : 139681
Request Date : 23/08/2021
Verification Date : 30/08/2021
Financial Category : نفقة الدولة - اورام دائر السلام
Department : Out Patient

Pathology Report

Clinical Data:

Rt breast cancer & Rt axillary lymphadenopathy (FNA done)

Nature of the specimen:

Slide revision

Gross Examination:

one block labeled as (5040/21), and 3 slides.

one block labeled as (982/21), and broken smear slides

Microscopic examination:

Sections examined reveals an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of moderately pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate.

cytologic examination of the smear slide and slide of cell block revealed positivity for malignancy.

Diagnosis:

Rt breast mass & Rt axillary LN (FNA), slide revision:

invasive duct carcinoma GII.

cytology and cell block positive for malignancy.

- Biomarkers:

- o ER: strong nuclear reaction in 60% of tumor cells (Allred score 7/8)
- o PR: moderate nuclear reaction in 10% of tumor cells (Allred score 4/8)
- o Her2: strong complete membrane reaction in > 10% of tumor cells (score 3+)

~~Prof. Amal Hareedy~~
~~Dr. Rana Fathy~~



Patient Name: Hanan Shaaban Hassan Othman	Date : 06/02/2024
Exam : MAMMOGRAPHY BOTH	Age : 53 Year(s)
Medical ID: 327/2021	Pat. ID: 22571

**BILATERAL BREAST MAMMOGRAPHY AND HIGH
RESOLUTION ULTRASOUND**

Indication:

Right breast conserving therapy

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)

Right breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Normal skin thickness.
- No regional lymphadenopathy

Left breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post right BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Best regards,
Dr. Eman Abdel Razik, MSc, FEBR
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCH, Cairo University

Handwritten signature and date: 6/2/2024



Patient Name: Hanan Shaaban Hassan Othman	Date : 03/11/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 52 Year(s)
Medical ID: 3027/2021	Pat. ID: 22571

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had right CBS in 6/2022

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific axillary LN, bilaterally with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Benign findings of the left breast (BIRADS 1)

Yours truly

Signature Dr. Engy Abdallah

03/11/2022 2:40 PM



Date : 06/02/2024	Patient Name: Hanan Shaaban Hassan Othman
Age : 53 Year(s)	Exam : ultrasound ABDOMEN& PELVIS
	Medical ID: 3027/2021

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size with homogenous echopattern
- No intra hepatic biliary radicle dilatation or focal lesions.
- Portal vein is patent

Gall Bladder: -

- Average distention with normal wall thickness with no stones.
- CBD is not dilated.

Spleen: -

- Average in size displaying homogenous echopattern, no focal lesions.

Kidneys: -

- Are of normal size , well differentiated & no gross stones or back pressure changes or space occupying lesion could be seen.

Para-aortic and pancreatic area: are concealed by gases.

No ascites or free fluid or collection seen .

Urinary Bladder: -

- Average filling with no stones or focal lesion seen

Opinion:

- Unremarkable study

Best regards
Dr/Shahd M. Fouad ,Msc

PATIENT NAME: HANAN SHABAN HASSAN AOSMAN	PATIENT ID #: 2021159864
GENDER: Female	ORDER #: 1146113,1146119
AGE: 50 y	DATE OF SERVICE: 25-Jul-2021 11:20 AM

Report: - all studies

**MULTI-SLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS
WITH TRIPHASIC STUDY OF THE LIVER REVEALED:**

(I) Chest:

- Right breast heterogeneously enhancing mass is seen measuring 2.6 cm.
- Prominent right axillary lymph nodes, the largest measuring 1.9X0.8 cm.
- Patent central air ways with well inflated both lungs.
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is average in size showing homogenous CT density and enhancement with no focal lesions.
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Bulky uterus.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Dorsolumbar spondylosis.

OPINION:

- **No CT evidence of metastatic pulmonary or hepatic metastatic disease.**

Reported By,

Dr. Amr Farouk ,MD
Dr. Mahmoud Saleh, MD

25-Jul-2021 02:52 PM



EAJN036749567

مردی فاضل

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
هرمل ، سابقا
إدارة الجودة

تذكرة العيادة الخارجية

التاريخ: ١٦ / ٢ / ١٤٣٨ هـ رقم القيد: ٥٠٩٧ الرقم القومي: ٩٦٩٦٩٦٩٦
اسم المريض: محمد بن عبد الله بن محمد بن عبد الله ابن:
الوظيفة: مدير عام شوي: الثاني ذكر
العنوان: الرياض
الهاتفون:
مجموعة: أ ب ج
توقيع كاتب الاختبار:

General Examination

vital signs:

ضغط الدم	درجة الحرارة	النبض	العمود	الوزن
توقيع التمريض				

Marital History: ☐ Married ☐ Single ☐ Widow ☐ Divorced
 Menopausal Status: ☐ Menopausal ☐ Premenopausal ☐ Postmenopausal
 Children: ☐ Yes ☐ No Number:
 Allergy: ☐ Yes ☐ No Comment:
 Adverse drug reaction: ☐ Yes ☐ No Comment:
 Smoking: ☐ Yes ☐ No Comment:
 Pain: ☐ Yes ☐ No Comment:

Complaints: _____

Present History:

Past History (hospital admission & surgery):

Family History:



Patient Name: Hanan Shaaban Hassan Othman	Date : 06/02/2024
Exam : MAMMOGRAPHY BOTH	Age : 53 Year(s)
Medical ID: 327/2021	Pat. ID: 22571

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- No sizable collection
- Normal skin thickness.
- No regional lymphadenopathy

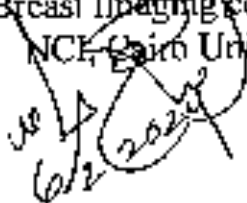
Left breast:

- No suspicious micro calcifications or architectural distortion detected.
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Opinion:

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Best regards,
Dr. Eman Abdel Rāzīk, MSc, FEBR
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCE, Cairo University


06/2/2024



Date : 06/02/2024	Patient Name: Hanan Shaaban Hassan Othman
Age : 53 Year(s)	Exam : ultrasound ABDOMEN& PELVIS
	Medical ID: 3027/2021

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Urinary Bladder:-

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Opinion:

- Unremarkable study

Best regards
Dr/Shahd M. Fouad ,Msc

PATIENT NAME: HANAN SHABAN HASSAN AOSMAN	PATIENT ID #: 2021158884
GENDER: Female	ORDER #: 1146113,1146119
AGE: 50 y	DATE OF SERVICE: 25-Jul-2021 11:20 AM

Report: - all studies

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- Bulky uterus.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Dorsolumbar spondylosis.

OPINION:

- No CT evidence of metastatic pulmonary or hepatic metastatic disease.

Reported By,

Dr. Amr Farouk, MD
Dr. Mahmoud Saleh, MD

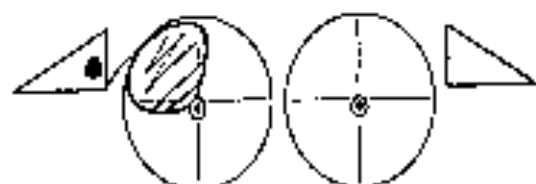
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مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السلطة المصرية
للحقوق
المرأة

٢٠٢١/٨/٢٤ MDT FORM

Name	حنان تيماني حسن
Age	
MRN	٢٧-١٥١٥٢٦-٩١٦٩
ID	302712021
Co morbidity	١٠١٦٥٠٤٤٠٥٥



Clinical Data

Bx:	IMC G-II	SM:	R+ UOQ multifocal suspicious enhancing mass postmen
IHC:		Mets Workup:	
ER:	718		Free
PR:	418		
Hert:	3+		
KITV:			

cT₂ N₁ M₀ cT

COMMENT:

- Bone scan
- Echo, labs
- Clip insertion

MDT OUTCOME & PLAN

- Brain MRI
- Neoadj Chemo AC x4 → Taxol/Hercophorin
+ Perjeta x4 → Consider ACS.B
repeated cell

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

14-2-2022

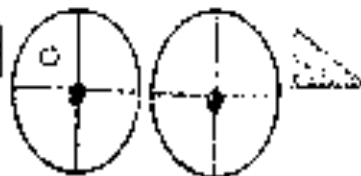


مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

المنشآت الصحية
مصر
طبعة
2020

MDT FORM

Name	هناء محمد
Age	31
MRN	00000000000000000000
ID	000000000000000000
Co morbidity	C.01 / 2.05



Clinical Data

Bx:	SM:
IHC: ER: 7/8 PR: 6/8 Her2: +++ Ki67:	Mets Workup:
cT3 N1 → yT1 N0 M0	
COMMENT: resect and LAC → 12 who Tumor + 4 x the cystic + Pseudo for CESM to assess R Conservation GCS	

MDT OUTCOME & PLAN

for BCS BCS + SLN (≥ 4LN) After wire localisation GCS
--

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الصحة المصرية
صحة
مصر

MDT FORM

2022/3/2

Name	جاءة محمد	MRN	٢٠٥٧٥٠٥١
Age	54	ID	٥٧٠٤١٥٧٠٥١٢
Menopausal status		Phone	١٢٦٧٧٠٤٤٧٥
OCP		Marital status	
Co morbidity		Family history	

Clinical Data

Rx: TAC 2 II

ER: 7/9 PR: 4/8 HER 2: ++ Ki67:

Sonomammography:

Mets Workup:

Free

CT INI

→ yT.NoM. (PCR)

Recommendations:

Acad to Rt Breast. post BC+I live

post RACT pathology: → pathological complete response SLNB → o/s pTo pNo

for post operation re MDT

MDT OUTCOME & PLAN

- Adj Trastuzumab SC. x 13 doses
- PORT to Rt. Breast & SC
- Adj hormonal th Arimidex x 54

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY



Med. Record No.: 2021158854
Patient Name: HANAN SHABAN HASSAN AQSMAN
Sex: Female DOB: 12/15/1970
Nationality: EGYPTIAN
Unit: External Clinic

SURGICAL PATHOLOGY - Final Anatomical Diagnosis
Biopsy Small - (1 Site)

Specimen Id	2107060250	Sample Type	Tissue
Accession No	SUR21-S040	Requesting Doctor	Abd ALLAH Mohamed Gamil
Collection Date	7/6/2021 12:43:04PM		
Receive Date	7/6/2021 12:43:12PM		

Clinical Information

US guided biopsy from RL breast UOQ suspicious lesion (BIRAD 4C)

Gross Picture

Multiple cores range from 4 up to 8 mm. Totally embedded in one block

Microscopic Picture

A case of invasive duct carcinoma, NST, grade II. There is associated intermediate grade DCIS (solid) pattern, constituting 7% of tumor focus. Stroma shows moderate inflammatory response. Lymphovascular emboli are seen.

Diagnostic Summary

Tumour type:

Invasive carcinoma, NST, grade II.

Laterality

Right

Location

Upper outer quadrant

Specimen type:

Core biopsy.

Histological grade

Mitosis score (1-3):

2

Tubular formation score (1-3):

3

Nuclear grade (1-3):

2

Histological grade (1-3):

2



Med. Record No.: 2021159864
Patient Name: HANAN SHABAN HASSAN AQSMAN
Sex: Female DOB: 12/15/1970
Nationality: EGYPTIAN
Unit: External Clinic

CYTOPATHOLOGY - Final Anatomical Diagnosis
(Cytology - 1 Site) فحص خلوي (1 Site)

Specimen Id: 2107050254 Sample Type: Fluid/ Smear
Accession No: CYTO21-982 Requesting Doctor: Abd ALLAH Mohamed Gamil
Collection Date: 7/6/2021 12:43:07PM
Receive Date: 7/6/2021 12:43:18PM

Nature of Specimen:

LUS guided FNAC from Rt axillary lymph node.

Gross Picture:

Four sections were prepared and Pap stained. One cell block was prepared.

Microscopic Picture:

Cell block and smears show scattered groups of malignant epithelial cells. The groups show glandular differentiation. Pleomorphism is moderate. Picture is of metastatic adenocarcinoma of possible breast origin.

Diagnosis:

Rt axillary lymph node, LUS guided biopsy:
Positive for metastatic adenocarcinoma, consistent with primary breast origin.

Comments:

For clinico-radiologic correlation.

Category Code (LN)

[C8: Malignant]

Signature

[AYA MOHAMMED ISMAIL]

Co-Signed by:

[GIHAD MOHAMED MD PhD]

Peritumoural lymphovascular invasion
Absent

Diagnosis

Rt breast UQC mass, US guided biopsy:
Invasive duct carcinoma, NST, grade 1.

Comments

ER, PR and HER2 were ordered

Biopsy reporting category

BSI= Invasive cancer

Signature

AYA MOHAMMED ISMAIL

Co-signed by:

[GHADA MOHAMED MD PhD.]

completed on: 7/14/2021

Referring Specialty :

Radiology

Type of Request :

Regular

History, Relevant Clinical Findings & Previous Inv. Results:

breast

Previous Pathology Result:

Procedure Done :

Site of Procedure Done:

Side of Procedure Done:

Right

Nature & Number of Specimens:

Reason for Order:

Surgical Pathology Exam

PATIENT NAME: HANAN SHABAN HASSAN AOSMAN	PATIENT ID #: 2021159884
GENDER: Female	ORDER #: 1131295,1131910
AGE: 50 y	DATE OF SERVICE: 08-Jul-2021 09:16 AM

Report: - all studies

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right breast suspicious mass lesion (FNAC outside revealed malignant cells).

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO view.
- Complementary ultrasound

REPORT:

- Bilateral moderate heterogenous back ground parenchymal enhancement with:
 - Right UOQ intensely enhancing mass lesion measuring 27.6x24mm (index lesion).
 - Another right UOQ two smaller enhancing mass lesions measuring 20x14.5mm and 10x6.5mm, all measuring an area of 92x30mm away from the nipple by 66.5mm (measured on CC view).
 - Right UOQ faint non mass enhancement measuring 41x32mm, collectively with the masses measuring an area of 93x95mm away from the nipple by 66mm.
 - No significant left enhancing lesions.
- Complementary Ultrasound showed right UOQ 9 o'clock three ill defined hypoechoic solid mass lesions measuring 26x25.3mm, 14.5x6.6mm and 8.3x7.6mm respectively, the nearest from the nipple at 25mm.
- Another right 10 o'clock ill defined Isoechoic lesion (corresponding to the non mass enhancement) measuring 22.5x12.2mm.
- Normal skin thickening and breast contour.
- Normal nipple areola complex.
- Right indeterminate axillary lymph nodes.

OPINION:

- Right breast UOQ multifocal suspicious enhancing mass lesions as described (BIRADS 4c) for histopathological correlation.

- No left pathological mass or non mass enhancement (BIRADS1).

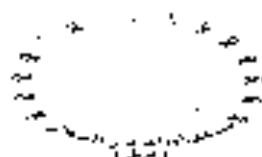
Recommendation - all studies

Referral to clinic

Reported By,

Dr. Mai Magdy, MSc

06-Jul-2021 11:33 AM



Name : Hanan Shaban Hassan

Date : 30-03-2021

MRI EXAMINATION OF THE BRAIN

Technique of examination:

- Axial T1, T2 & Flair WIs "Plate 1-3".
- Sagittal T1 WIs "Plate 4".
- Post contrast axial sagittal and coronal T1 WIs "Plates 5,6,7".

A known case of breast cancer.

MRI Findings:

- Normal size shape & position of the cerebral ventricles with no midline shift or deformity.
- Normal gray–white matter interface with no areas of abnormal signal intensity.
- No evidences of mass lesions.
- No areas of abnormal enhancement are seen at the post contrast scan.
- No gross vascular abnormality.
- Normal cerebellum, brain stem & cervico – medullary junction.
- Normal sellar region.
- Normal extra axial spaces.

Opinion:

- *Normal contrast enhanced MRI study of the brain.*

Dr. Reham Anwar, MSc. FRCR

Dr m.gumaa

Clinical Summary

Right Breast IDC Grade II
Pre & Post contrast Mediolateral oblique and CC views
Targeted High resolution ultrasound

Detailed Findings

Left breast:
• Dense fibroglandular parenchyma. (ACR C)
• Contrast enhanced images demonstrated
• No mass or non-mass enhancement.
• Normal skin/nipple/areolar complex.
• No pathological axillary node.

Right breast:

• Dense fibroglandular parenchyma. (ACR C)
• Pre-contrast images revealed the UOQ ill-defined mass in
the underlying lesions, located at 9-10 o'clock, largest measuring about
15x7mm.
• Normal skin/nipple/areolar complex.
• No definite pathological axillary lymph node seen.

Conclusion:

• Right breast: Residual UOQ multifocal process, showing no
enhancement (BIRADS 6).
• Left breast: normal study (BIRADS 1).

Best regards,

Dr. Mahi Mokhtar

Name: Hanan Shabaan Hassan

51Y

F

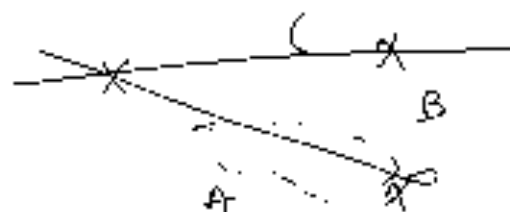
Date: 24/2/2022

Wire localization for right breast masses:

- Under local anesthesia, antiseptic condition and ultrasound guidance.
- Ultrasound guided wire localization was inserted into the right breast 10 o'clock residual.
- With the following distances:
 - Whole distance of the wire till the tip: 3.2 cm
 - Perpendicular distance from skin to tip: 1.2 cm
 - Distance from perpendicular distance to skin entry: 2.9 cm.
- Pre operative breast wire localization.

Marker was placed on the skin over the tip as well as bracketing of the distal extension of the parenchymal distortion.

**Best Regards,
Dr. Ahmed M. Rozeka, MSc**



A = 3.2 cm

B = 1.2 cm

C = 2.9 cm

Dr. Ahmed
 Rozeka
 24/2/2022



Patient Name: Hanan Shaaban Hassan Othman	Date : 03/11/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 52 Year(s)
Medical ID: 3927/2021	Pat. ID: 22571

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had right CBS in 6/2022

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific axillary LN, bilaterally with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Benign findings of the left breast (BIRADS 1)

Yours truly

Signature Dr. Engy Abdallah

03/11/2022 2:40 PM



8/11

المرضى
المرضى
المرضى

حنا شهاب حسن

ort

19

اسم المريض :
رقم الملف :
نوع التوجيه :
تاريخ اعداد التوجيه :
وقت اعداد التوجيه :

EF	61	55-75%	PW1	0.7	0.6-1.1cm
LVEDD	5.3	3.5-5.8cm	AOR	2.9	2-3.7cm
LVEDS	3.5	2-4cm	LA	3.9	2-4cm
FS	33	25-45%	RV	not	0.7-2.3cm(supine)
SWT	0.7	0.6-1.1cm	AO.CUSP	not	1.5-2.6cm

LV dimensions

LV wall thickness

LV diastolic function

SWMA

LA and Ao root dimensions

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion

MSC

Echo report

Name: *Mohammed Anwar*

Date: 24/8/2021

EF	70	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	5.1	3.5-5.8cm	AOR	3.0	2-3.7cm
LVESD	3.1	2.4cm	LA	3.6	2-4cm
FS	35	25-45%	RV	—	0.7-2.3cm (supine)
SWT	0.9	0.6-1.1cm	AO.CUSP	—	1.5-2.5cm

LV dimensions *normal*

LV wall thickness *normal*

LV diastolic function *GI DD*

SWMA *normal*

LA and Ao root dimensions *normal*

Mitral Valve *thickened & grade II MR*

Aortic valve *normal*

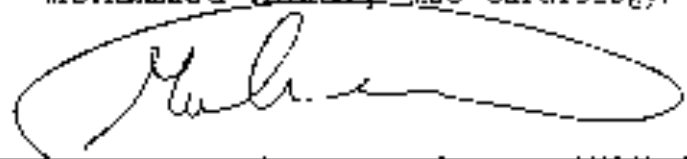
Tricuspid valve *normal & Trivial TR, RVIP = 19 mm*

Pulmonary valve *normal*

Right side and systolic function *normal*

Intracardiac masses and effusion *normal*

Mohammed Anwar, MSc cardiology.



Name	حنان شعبان حسن	Age, Sex	51 Yrs, F
Referred by	Prof. Dr:	Date	23 / 11 / 2021

Transthoracic Echocardiography Report

M- Mode measurements:

	Patient	Normal range		Patient	Normal range
LVED (mm)	52.2	M 42 - 58 F 38 - 52	Ao. Root Annulus(mm)	24	M 23 - 29 F 21 - 25
LVES (mm)	32	M 28 - 40 F 27 - 35	LA (mm)	37.5	M 30 - 40 F 27 - 38
IVS (mm)	9.1	6 - 10	RV Basal diameter(mm)	32	25 - 41
LVPW (mm)	8.7	6 - 11	EF (%)	68	M 52 - 72 F 54 - 74

Comment:

- Normal sinus rhythm, with good echogenicity.
- Normal left ventricular internal dimension with normal wall thickness.
- Good left ventricular systolic function with estimated EF 68 %.
- Normal diastolic function.
- No resting segmental wall motion abnormalities.
- Normal left atrium dimension.
- Normal right ventricle structures and function.
- Mild mitral regurge.
- Normal other cardiac valves morphology and function.
- No pulmonary hypertension.
- No pericardial effusion.
- No apparent intracardiac masses or thrombi.
- Intact interatrial and interventricular septae.

Conclusion:

- Mild MR

Dr. Yaser Gamal
Cardiology specialist

Name	حنان شعبان حسن	Age, Sex	51 Yrs, F
Referred by	Prof. Dr:	Date	22 / 2 /2022

Transthoracic Echocardiography Report

M- Mode measurements:

	Patient	Normal range		Patient	Normal range
LVED (mm)	49.5	M 42 – 58 F 38 – 52	An.Ront Annulus(mm)	24	M 23 – 29 F 21 – 25
LVES (mm)	31.4	M 25 – 40 F 22 – 35	LA (mm)	36.8	M 30 – 40 F 27 – 38
IVS (mm)	8.3	6 – 10	RV Basal diameter(mm)	32	25 – 41
LVPW (mm)	8.3	6 – 10	EF (%)	66	M 52 – 72 F 54 – 74

Comment:

- Normal sinus rhythm, with good echogenicity.
- Normal left ventricular internal dimension with normal wall thickness.
- Good left ventricular systolic function with estimated EF 66 %.
- Normal diastolic function.
- No resting segmental wall motion abnormalities.
- Normal left atrium dimension.
- Normal right ventricle structures and function.
- Mild mitral regurge.
- Normal other cardiac valves morphology and function.
- No pulmonary hypertension.
- No pericardial effusion.
- No apparent intracardiac masses or thrombi.
- Intact interatrial and interventricular septae.

Conclusion:

- Mild MR.

Dr. Yaser Gamal
Cardiology special

Echo report

Name: *[Signature]*

Date: 2/1/2021

EF	62	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.2	3.5-5.8cm	AO R	3.8	2-3.7cm
LVEDS	2.7	2.4cm	LA	3.8	2-4cm
FS	33	25-45%	RV		0.7-2.3cm (supine)
SWT	0.9	0.6-1.1cm	AO CUSP		1.5-2.6cm

LV dimensions *Normal*

LV wall thickness *Normal*

LV diastolic function *Normal*

SWMA *Normal*

LA and Ao root dimensions *Normal*

Mitral Valve *Normal*

Aortic valve *Normal*

Tricuspid valve *Normal*

Pulmonary valve *Normal*

Right side and systolic function *Normal*

Intracardiac masses and effusion *No masses or effusion*

[Signature]
1/2/2021



Patient Name: *Hanan Shaaban Hassan.*

Date: 14/10/2021

Tc-99m MDP bone scan.

Desired information : skeletal surveying to detect multiple lesions

Technique:

Delayed Anterior and posterior whole body scan 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP.

Delayed Static Phase:-

- Degenerative changes are noticed over dorsal and lumbar vertebrae for follow up.
- Otherwise the rest of surveyed skeleton is free from serious bone lesions.
- Both kidneys are seen.

Conclusion:

No definite evidence of metastatic osseous deposits.

Follow up bone scan is advised.

Thanks for reference

Nuclear medicine Unit

Huda Fathy , M.D



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السلطة الصحية
مصر

MDT FORM

1-78678549

21/3/2022

Name	فايزة محمد	MRN	٢٠٩٧٠٩٠٩١
Age	51y	ID	٩٧٠٩٧٠٩٧٠٩٧٠
Menopausal status		Phone	٠١١٧٧٧٧٧٧٧٧
DCP		marital status	
Comorbidity		Family history	

Clinical Data

Ex:	INC 2 II	R	
EP:	7/1	PR:	4/1
HER2:	+++	Ki67:	
Sonomammography:			
Mets Work up	Free		
		CT 3M	
		→ yPToNoMo	CPR

Recommendations: A case of Rt Breast post BC+ST
post RACT pathology: → pathological complete
response SLNB → 0/5 pT0 pN0
for post-operative re MDT

MDT OUTCOME & PLAN

- Pr - Adj Trastuzumab SC. x 13 doses
- PORT to Rt. Breast 2 SC
- Adj hormonal th Arimidex x 54

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

16 - 2 - 2022

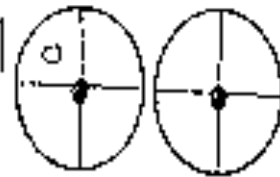


مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنيت المصرية
صحة
مصر

MDT FORM

Name	هناء محمد
Age	51
MRN	5.11.151057.5177
ID	1111774440
Co morbidity	C.51 / 4.5V



Clinical Data

Bx:	SM:
IHC: ER: 7/8 PR: 4/8 Her2: ++ KITV:	Mets Workup:
cT3 N1 → 4T1 N0 M0	
COMMENT: received LAC → 12 wk Trial + 4 x Herceptin + Plavix for CESM to a MR L Guidance	

MDT OUTCOME & PLAN

for BCS BCS + SLN (≥ 4 LN) After wire localisation
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SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY



Med. Record No.: 2021158854
Patient Name: HANAN SHABAN HASSAN AOSMAN
Sex: Female DOR: 12/15/1973
Nationality: EGYPTIAN
Unit: External Clinic

IMMUNOPATHOLOGY - Final Anatomical Diagnosis

Hormone receptors

Specimen Id 2107100020

Sample Type

Paraffin Block

Accession No IMM21-4204

Requesting Doctor

Abd ALJAF, Mohamed Gamil /

Collection Date 7/10/2021 8:54:08AM

Receive Date 7/10/2021 8:54:13AM

Immunohistochemical Report

pathology number

504023

Clinical Data

RT, breast cancer

Gross Picture :

Referred block submitted for ER/PR immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer

Antibody:

Ventana anti-PR(1E2) Rabbit monoclonal antibody, Ventana anti-ER(SP1) Rabbit monoclonal antibody.

Scoring system :

According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive. Allied assay: proportion of cells staining (0, no staining; 1, < 1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%; and 5, between 67%–100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8
Comment: - There are much more limited data on the 2% to 3% of cancers that are low ER expression (most often defined as 1%-10% ER-positive cells by IHC) and the potential benefit from endocrine therapy. - PR positive in 1-20% of tumor cells is considered as low positive.

(Laterality) breast Cancer:

Right

Diagnosis :

ER: Positive, intermediate nuclear reaction in 70% of tumor cells (7/8)
PR: Low Positive, intermediate nuclear reaction in 5% of tumor cells (4/8).

Signature

GHADA MOHAMED MD PHD.



11/93

وزارة الصحة
وزارة الداخلية
مستشفى الامام
مركز اورام دار السلام
قسم القلب والرئتين
قسم الموجات فوق الصوتية
تاريخ اعداد الموجة
رقم اعداد الموجة

وزارة الصحة والامكان
امانة المراكز الطبية المتخصصة
مركز اورام دار السلام

حبيب محمد

eport

Date: / / 2021

EF	60	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	5.2	3.5-5.8cm	AO R	3.0	2-3.7cm
LVEDD	3.5	2-4cm	LA	3.7	2-4cm
FS	32	25-45%	RV	normal	0.7-2.3cm(supine)
SWT	1.1	0.6-1.1cm	AO CUSP	normal	1.5-2.6cm

LV dimensions normal

LV wall thickness normal

LV diastolic function normal

SWMA normal

LA and Aortic root dimensions normal

Mitral Valve thickened & trivial MR

Aortic valve normal

Tricuspid valve normal & trivial TR, normal R.V.P.

Pulmonary valve normal

Right side and systolic function normal

Intracardiac masses and effusion normal

D mohammed Anwar

Msc cardiology



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقا)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :
الرقم الطبي :

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

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التاريخ: ٢٠٢٠/٢/١٦
الوقت:

توقيع الطبيب : د. / رهنه مصطفى

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

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التاريخ: ٢٠٢٠/٢/١٦
الوقت:

التاريخ: ٢٠٢٠/٢/١٦
الوقت:

تامو وديت ٢

كالسيوم

وانا العنا

نيرونيت

بابا دولا ٥-٥ كل ٤ ساعات

النا ايس كبريت

تم الوريد
٢٠٢٠/٢/١٦

توقيع الطبيب



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض:
الرقم الطبي: ٢٠٢٤/١١/٢٤

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

2/24 2/24 2/24
1/9/24 2/24 2/24

توقيع الطبيب:
تاريخ: 2/24

Chest:

Temp:

Oral mucositis:

Canula site infection:

24/11/2024

توقيع الطبيب:
تاريخ: 24/11/2024

التاريخ:

الوقت:

24/11/2024
11:52
نور

24/11/2024



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :
الرقم الطبي : ٣٠٩٧ / ٥٠١

الأوامر

Chest:
Temp:
Oral mucositis:
Canula site infection:

6/2/2024
أوامر الطبيب :
1- متابعة المريض
2- متابعة المريض
3- متابعة المريض
4- متابعة المريض
5- متابعة المريض
6- متابعة المريض
7- متابعة المريض
8- متابعة المريض
9- متابعة المريض
10- متابعة المريض

التاريخ :
الوقت :

توقيع الطبيب :
الاستاذ
د. السيد
١٤٣٦

الأوامر

Chest:
Temp:
Oral mucositis:
Canula site infection:

أوامر الطبيب :
1- متابعة المريض
2- متابعة المريض
3- متابعة المريض
4- متابعة المريض
5- متابعة المريض
6- متابعة المريض
7- متابعة المريض
8- متابعة المريض
9- متابعة المريض
10- متابعة المريض

التاريخ :
الوقت :

توقيع الطبيب :
الاستاذ
د. السيد
١٤٣٦

**Doctor's Orders** **أوامر الطبيب**

الأوامر

توقيع الطبيب: _____

ملاحظات

التاريخ: ٠٢٤/٦/١٤
(وقت:)

ولفيع الحبيب



Echo:
(23|11|222) $\rightarrow$ 60%.

Doctor's Orders **أوامر الطبيب**

اسم المريض:
الرقم الطبي: ٢٠٢١/٣-٤

[illegible][illegible]



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقا)
ادارة الجودة

Doctor's Orders **أوامر الطبيب**

اسم المريض :

ترقيم التعلیقات

الأوامر

chest:

Temp: _____

Oral mucositis:_____

Canula site infection... 18

C44

2.5

ion.  

$$Y = X \in \mathbb{R}^1 \quad (\text{no restriction})$$
$$\lambda' x \in X \quad \text{cp} \quad \downarrow \quad \text{red}$$

1. 学习

- Y. R. C. M. 14. 1. 6

توقيع الطبيب :

Cluster

Q. 2. 2. 2.

1211

الأوامر

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chest: 0123456789ABCDEF0123456789ABCDEF0123456789ABCDEF0123456789
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Temp:.....

Oral mucositis:

Capula site infection.....

3/11/2022

P. L. X. L. (1911) 5

10X444 sec 4.44

1 2 3 4 5 6 7 8 9 10 11 12 13 14 15 16 17 18 19 20 21 22 23 24 25 26 27 28 29 30 31 32 33 34 35 36 37 38 39 40 41 42 43 44 45 46 47 48 49 50 51 52 53 54 55 56 57 58 59 60 61 62 63 64 65 66 67 68 69 70 71 72 73 74 75 76 77 78 79 80 81 82 83 84 85 86 87 88 89 90 91 92 93 94 95 96 97 98 99 100 101 102 103 104 105 106 107 108 109 110 111 112 113 114 115 116 117 118 119 120 121 122 123 124 125 126 127 128 129 130 131 132 133 134 135 136 137 138 139 140 141 142 143 144 145 146 147 148 149 150 151 152 153 154 155 156 157 158 159 160 161 162 163 164 165 166 167 168 169 170 171 172 173 174 175 176 177 178 179 180 181 182 183 184 185 186 187 188 189 190 191 192 193 194 195 196 197 198 199 200 201 202 203 204 205 206 207 208 209 210 211 212 213 214 215 216 217 218 219 220 221 222 223 224 225 226 227 228 229 230 231 232 233 234 235 236 237 238 239 240 241 242 243 244 245 246 247 248 249 250 251 252 253 254 255 256 257 258 259 260 261 262 263 264 265 266 267 268 269 270 271 272 273 274 275 276 277 278 279 280 281 282 283 284 285 286 287 288 289 290 291 292 293 294 295 296 297 298 299 300 301 302 303 304 305 306 307 308 309 310 311 312 313 314 315 316 317 318 319 320 321 322 323 324 325 326 327 328 329 330 331 332 333 334 335 336 337 338 339 340 341 342 343 344 345 346 347 348 349 350 351 352 353 354 355 356 357 358 359 360 361 362 363 364 365 366 367 368 369 370 371 372 373 374 375 376 377 378 379 380 381 382 383 384 385 386 387 388 389 390 391 392 393 394 395 396 397 398 399 400 401 402 403 404 405 406 407 408 409 410 411 412 413 414 415 416 417 418 419 420 421 422 423 424 425 426 427 428 429 430 431 432 433 434 435 436 437 438 439 440 441 442 443 444 445 446 447 448 449 450 451 452 453 454 455 456 457 458 459 460 461 462 463 464 465 466 467 468 469 470 471 472 473 474 475 476 477 478 479 480 481 482 483 484 485 486 487 488 489 490 491 492 493 494 495 496 497 498 499 500 501 502 503 504 505 506 507 508 509 510 511 512 513 514 515 516 517 518 519 520 521 522 523 524 525 526 527 528 529 530 531 532 533 534 535 536 537 538 539 540 541 542 543 544 545 546 547 548 549 550 551 552 553 554 555 556 557 558 559 560 561 562 563 564 565 566 567 568 569 570 571 572 573 574 575 576 577 578 579 580 581 582 583 584 585 586 587 588 589 590 591 592 593 594 595 596 597 598 599 600 601 602 603 604 605 606 607 608 609 610 611 612 613 614 615 616 617 618 619 620 621 622 623 624 625 626 627 628 629 630 631 632 633 634 635 636 637 638 639 640 641 642 643 644 645 646 647 648 649 650 651 652 653 654 655 656 657 658 659 660 661 662 663 664 665 666 667 668 669 670 671 672 673 674 675 676 677 678 679 680 681 682 683 684 685 686 687 688 689 690 691 692 693 694 695 696 697 698 699 700 701 702 703 704 705 706 707 708 709 710 711 712 713 714 715 716 717 718 719 720 721 722 723 724 725 726 727 728 729 730 731 732 733 734 735 736 737 738 739 740 741 742 743 744 745 746 747 748 749 750 751 752 753 754 755 756 757 758 759 760 761 762 763 764 765 766 767 768 769 770 771 772 773 774 775 776 777 778 779 780 781 782 783 784 785 786 787 788 789 790 791 792 793 794 795 796 797 798 799 800 801 802 803 804 805 806 807 808 809 810 811 812 813 814 815 816 817 818 819 820 821 822 823 824 825 826 827 828 829 830 831 832 833 834 835 836 837 838 839 840 841 842 843 844 845 846 847 848 849 850 851 852 853 854 855 856 857 858 859 860 861 862 863 864 865 866 867 868 869 870 871 872 873 874 875 876 877 878 879 880 881 882 883 884 885 886 887 888 889 890 891 892 893 894 895 896 897 898 899 900 901 902 903 904 905 906 907 908 909 910 911 912 913 914 915 916 917 918 919 920 921 922 923 924 925 926 927 928 929 930 931 932 933 934 935 936 937 938 939 940 941 942 943 944 945 946 947 948 949 950 951 952 953 954 955 956 957 958 959 960 961 962 963 964 965 966 967 968 969 970 971 972 973 974 975 976 977 978 979 980 981 982 983 984 985 986 987 988 989 990 991 992 993 994 995 996 997 998 999 1000 1001 1002 1003 1004 1005 1006 1007 1008 1009 1010 1011 1012 1013 1014 1015 1016 1017 1018 1019 1020 1021 1022 1023 1024 1025 1026 1027 1028 1029 1030 1031 1032 1033 1034 1035 1036 1037 1038 1039 1040 1

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الموقع الجغرافي :

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التاريخ:

الوقت:

Echo Req. before
next cycle



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض : حمادة شعيب
الرقم الطبي :

الأوامر

Chest:
Temp:
Oral mucositis:
Canula site infection:

C12 Herceptin 500 mg SC

for Tamoxifen Tab 1x2 x3u

المريضة صرقت شهر واحد فقط

التاريخ: ٢٠٢٢ / ٨ / ٣١
الوقت:

توقيع الطبيب : د. الشياح عبد السلام

C12 Her
aph Aya
21/8

الأوامر

Chest:
Temp:
Oral mucositis:
Canula site infection:

المريضة صرقت شهر واحد فقط

for Tamoxifen Tab 1x2 x3u

المريضة صرقت شهر واحد فقط

for Tamoxifen Tab 1x2 x3u

المريضة صرقت شهر واحد فقط

التاريخ: ٢٠٢٢ / ٨ / ٣١
الوقت:

C12 Her
aph Aya
21/8



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :
الرقم الطبي :

الأوامر

Chest:

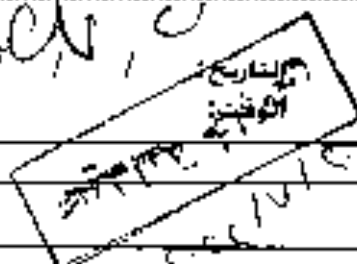
Temp:

Oral mucositis:

Canula site infection:

Herceptin 600 mg SC
9/24/12
on Ex. Free

CCN, C



C10 Herceptin
cl.ph
2017/2012

د/كريم

توقيع الطبيب :

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

EF = 61% 10/8/12

C11 Herceptin 600 mg SC

C11 H
aphaga
10/16

التاريخ: 10/1/12
الوقت:

د/كريم

توقيع الطبيب :



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقا)
إدارة الجودة

Doctor's Orders أوامر الطبيب

EG 621

اسم المريض

26/4/2022

الرقم الطبي

الأوامر

chest:

Temp:

Oral mucositis:

Canula site infection:

C6 هيرسيته

C. جاسمرازول

C1 x 3

نيروفين

C6H

.....

Oph Aya

0.00/1/19

26/4/2022

مراجعة كاتر

توقيع الطبيب

EG 621

02

الأوامر

chest:

Temp:

Oral mucositis:

Canula site infection:

احصاء ٢٠٠٠ بالماء والصابون

تليد استكن بالصابون

Hal 141

توقيع الطبيب

القنينة

4.1/2/15

التاريخ:

الوقت:



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض:
الرقم الطبي:

51 سنة

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

ER 7/8 - PR 4/8

K67

HER +++

Rt Breast Ca

→ P.T. No Mo → PCR achieved

For adjuv: trastuzumab 600 mg sc

X 13 doses

توقيع الطبيب:

التاريخ:
الوقت:

أحمد حسين محمد حسن

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

ophth. B

8/5/4/2022

X X X X X X X X X X

سجل
دلتا

5/2/0

توقيع الطبيب:

التاريخ:
الوقت:



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :

الرقم الطبي :

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

SI 4 e anal abscess (chronic)?

for your kind care

1/19/99

توقيع الطبيب :

التاريخ :

الوقت :

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

patient complains of peripheral neuropathy

RI Neurovit tab 1x1x3

RI Gafin 300mg 1x2x3

توقيع الطبيب :

التاريخ :

الوقت :



مركز اخصية المتخصصة
مستشفى اورام دار السلام (هرمس)
الصيدلة الاكلينيكية

TC₄HP (with weekly Paclitaxel) cycle (.....)

التاريخ: 2022/...../.....			
اسم المريض: رقم الملف:			
Weight: kg	Height: cm	BSA: M2	Age: years
CBC:	WBC:	ANC:	Hgb: PLT:
Functions Tests:	Sr.Creat:	Urea:	CrCL:
	ALT:	AST:	T.Bil: D.Bil:
Echo:	Date:	EF: %	
Dose Modification: %			
Reason: ☐ Hematological ☐ Non hematological:			
امضاء التمريض:	بروتوكول العلاج		المسرف
الاسبوع الاول:	جرانيسرون (0.01 مجم/كجم) مجم + ديكساميثازون 20 مجم + لتودين 20 مجم + امبول افيل) في 50 مل ح 5% على مدى ربع ساعة قبل العلاج الكماوي. (Maximum dose of Granisetron = 1mg)		الاسبوع 1: بتاريخ 19/...../.....
الاسبوع الثاني:	بالترياكسيل (80 مجم/2) 1.5.2 مجم في 250 مل ح 5% على مدى ساعة.		بتاريخ 19/...../.....
الاسبوع الثالث:	كازوبلاتين (AUC3) مجم على مدى ساعة		الاسبوع 1: بتاريخ 19/...../.....
	تراستوزوماب 600 مجم تحت الجلد لمدة 5 دقائق في الجلد بروتوزوماب (7.7) مجم في 250 مل ملح على مدى 1/2 ساعة (على مدى ساعة في الجرعة الاولى) Loading Dose: 840 mg Maintenance Dose: 420 mg		بتاريخ 19/...../.....
كورولوك قرص 40 مجم على تريق يوميا ديكساميثازون 8 مجم (امبول او قرص) مرة يوميا اليوم الثاني و الثالث. ☒ جرانيسرون قرص 1 مجم كل 12 ساعة اليوم الثاني و الثالث. ☐ اوندالينز 8 مجم قرص كل 12 ساعة اليوم الثاني و الثالث. فلاجيل 500 1 3 2 1			
Physician Signature:	Clinical pharmacy:		Dispensing pharmacy:
.....	W10 >> Taxol /C ₄ HP 19/11/2022 W11 >> Taxol 28/11/2022 W12 >> Taxol Dose B 2/2 trastuzumab + pertuzumab > Date		



Ministry OF Health
Dar El-Salam Cancer Hospital
Clinical Pharmacy Team

ph: Aya Fayed
12/11/2021
W7 (taxol)

توفد الجيرة القائمة
بتحاليل جديدة

ADW 12/11/2021
12/11/2021
12/11/2021

TCHP (with weekly Taxol) cycle (.....)

التاريخ: 2021 / /
المرضى:
اسم المريض:
Weight: 87 kg Height: 155 cm BSA: 1.9 M2 Age: years
CBC: WBC: ANC: Hgb: PLT:
Functions Tests: Sr.Creat: Urea: CrCL:
ALT: AST: T.Bil: D.Bil:
Echo: Date: EF: %

Dose Modification:

Reason: ☐ Hematological ☐ Non hematological:

امضاء التمريض	بروتوكول العلاج	انصرف
الاسبوع الاول:	(جر انقريب (D.01 مج/كجم) مج + نيكسيميثازون 20 مج + ستونين 20 مج + امبول في 50 مل ج 5% على مدى ربع ساعة قبل العلاج الكيمائي. (Maximum dose of Granisetron = 1mg)	الاسبوع 1 بتاريخ 12/11/2021
الاسبوع الثاني:	تفصول (80 مج/2) 15.2 مج في 250 مل ج 5% على مدى ساعة	الاسبوع 2 بتاريخ 13/11/2021
الاسبوع الثالث:	تفصول (80 مج/2) 15.2 مج في 250 مل ج 5% على مدى ساعة	الاسبوع 3 بتاريخ 14/11/2021
	هرميتين 600 مج تحت الجلد لمدة 5 دقائق في الفخذ بريفينا 250 مج في 250 مل منع على مدى 1/2 ساعة (على مدى ساعة في الجرعة الاولى) Loading Dose: 840 mg Maintenance Dose: 420 mg	يعطى بتاريخ / /

EF 88%
23/11/2021

العلاج التدايمي:
هيدروكورتيزون 40 مج على طريق يوميا
نيكسيميثازون 8 مج (امبول او قرص) مرة يوميا اليوم الثاني والثالث
جر انقريب قرص 1 مج كل 12 ساعة اليوم الثاني والثالث
لووندايفز 8 مج قرص كل 12 ساعة اليوم الثاني والثالث

Physician Signature:
Clinical pharmacy:
Dispensing pharmacy:
W7: 12/11/2021
W8: 12/11/2021
W9: 12/11/2021
Herceptin + perjeta Date: 12/11/2021

12/11/2021



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :

الرقم الطبي :

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

Handwritten notes in Arabic: "والله أعلم" (In the name of God, I know), "P.X", and "C.X".

التاريخ: / /

الوقت: / /

توقيع الطبيب:

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

Handwritten notes in Arabic: "رغمه", "P.X", "C.X", and "Send to Proct".

التاريخ: / /

الوقت: / /

توقيع الطبيب:

Handwritten signature and date: "C. 11/10/2020".